

1. Administrative & Study Identification

Full Study Title: AN INTERVENTIONAL, OPEN-LABEL, RANDOMIZED, MULTICENTER PHASE 3 STUDY OF PF-07220060 PLUS LETROZOLE COMPARED TO CDK4/6 INHIBITOR PLUS LETROZOLE IN PARTICIPANTS OVER 18 YEARS OF AGE WITH HORMONE RECEPTOR (HR)-POSITIVE, HER2-NEGATIVE ADVANCED/METASTATIC BREAST CANCER WHO HAVE NOT RECEIVED ANY PRIOR SYSTEMIC ANTICANCER TREATMENT FOR ADVANCED/METASTATIC DISEASE
Short Title/Acronym: FourLight-3 **Protocol Number:** C4391024 **NCT Number:** NCT06760637 **Sponsor Name:** Pfizer **Company:** Pfizer
Investigational Product: PF-07220060 (atirmociclib) + letrozole
Phase of Development: Phase III **Trial Status:** RECRUITING **Medical Monitor:** Pfizer Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the efficacy of the investigational CDK4 inhibitor PF-07220060 (atirmociclib) in combination with letrozole compared to approved treatments (palbociclib, ribociclib, or abemaciclib plus letrozole) as measured by Progression Free Survival (PFS) by Blinded Independent Central Review (BICR). **Secondary Objectives:** To evaluate Overall Survival (OS), objective response, duration of response, clinical benefit rate, safety, patient-reported outcomes, and changes in circulating tumor DNA.

2.2 Background & Rationale To determine the safety and efficacy of PF-07220060 combined with letrozole compared to currently approved CDK4/6 inhibitors plus letrozole as a first-line treatment in adults with HR-positive, HER2-negative locally advanced or metastatic breast cancer. This study explores if a highly selective CDK4 inhibitor can provide superior or prolonged benefits while addressing acquired or intrinsic resistance seen in existing therapies.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator (Investigator's choice of standard approved therapy: abemaciclib, palbociclib, or ribociclib + letrozole)
Blinding: Open-label **Randomization:** Randomized (1:1 allocation)

3.2 Planned Enrollment & Duration Target \$N\$: 1,020 participants
Number of Sites: Global and Regional breakdown (Over 500 locations across North America, South America, Europe, Australia,

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Adults ≥ 18 years of age. 2. Histological confirmation of breast cancer with evidence of locally advanced or metastatic disease, which is not amenable to surgical resection or radiation therapy with curative intent. 3. Documented estrogen receptor (ER) and/or progesterone receptor (PR)-positive tumor, and documented HER2-negative tumor. 4. Previously untreated with any systemic anticancer therapy for locally advanced or metastatic disease. 5. Measurable disease or non-measurable bone-only disease as defined by RECIST version 1.1.

4.2 Exclusion Criteria 1. In visceral crisis at risk of immediately life-threatening complications in the short term. 2. Current or past history of central nervous system (CNS) metastases. 3. Have received prior (neo)adjuvant endocrine therapy (ET) and had recurrence during or within 12 months after the last dose of ET. 4. Have received prior (neo)adjuvant CDK4/6 inhibitor therapy and had recurrence during or within 12 months after the last dose. 5. Inadequate renal function, hepatic dysfunction, or hematologic abnormalities.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: PF-07220060 (atirmociclib) plus letrozole (Trade Name: Femara, ATC Code: L02BG04), dosage as defined by study protocol. **Route of Administration:** Oral **Duration of Treatment:** Ongoing continuous treatment until disease progression, unacceptable toxicity, or patient withdrawal.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care for cancer-related symptoms and co-morbidities. **Prohibited:** Concurrent administration of other systemic anti-cancer therapies or unapproved investigational agents.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
1	-28 to 0	Screening: Informed consent, medical history review, blood tests, physical exam, ECG, CT/MRI tumor scans to confirm eligibility.
2	Day 0	Baseline: Randomization (1:1), initial dispensing of PF-07220060 + letrozole or Comparator + letrozole.
3-12	Per Cycle	Interim: Regular clinic visits: Safety blood/urine tests, ECG monitoring, questionnaires, targeted therapy dispensing, and CT/MRI scans to

monitor tumor size and response. | | **End of Study** | At progression | Treatment discontinuation, final safety labs, and ongoing survival follow-up (up to approx 13 years). |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Progression Free Survival (PFS): Time from the date of randomization to the date of the first documentation of objective progressive disease or death due to any cause. **Measurement Method:** Assessed by Blinded Independent Central Review (BICR) per RECIST v1.1.

7.2 Secondary & Exploratory Endpoints * Overall Survival (OS) * Objective Response Rate (ORR) and Duration of Response (DoR) * Clinical Benefit Rate * Patient-Reported Outcomes (PROs) and safety metrics * Changes in circulating tumor DNA

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring via regular clinic visits, physical examinations, laboratory tests, and ECGs. **Serious Adverse Events (SAEs):** Reported by investigators per standard 24-hour reporting timelines. **Data Monitoring Committee (DMC):** Independent data monitoring committee to periodically review safety and efficacy data.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) Set for primary efficacy analysis; Safety Set for patients receiving at least one dose of the study drug. **Sample Size Justification:** A target $N=1,020$ is statistically powered ($\alpha = 0.05$, expected $Power \geq 80\%$) to determine the superiority of PF-07220060 + letrozole vs. standard CDK4/6i + letrozole for PFS. **Handling of Missing Data:** Standard survival analysis methods, censoring patients at last known progression-free date for the primary PFS endpoint.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Routine onsite and remote clinical monitoring visits to ensure protocol adherence and data integrity. **Audit Trail:** Utilizing secure electronic Case Report Forms (eCRF) with full audit trails and data clarification processes.

11. Study Identifiers & Governance

Primary ID: {{C4391024}} **Registry Number:** {{NCT06760637}} **Posting ID:** {{173690001021019368}} **Sponsor/Lead Organization:** {{Pfizer}} **Study Title (Brief):** {{Study of PF-07220060 With Letrozole in Adults With HR-positive HER2-negative Breast Cancer Who Have Not Received Anticancer Treatment for Advanced/Metastatic Disease}} **Acronym:** {{FourLight-3}} **Official Regulatory Title:** {{AN INTERVENTIONAL, OPEN-LABEL, RANDOMIZED, MULTICENTER PHASE 3 STUDY OF PF-07220060 PLUS LETROZOLE COMPARED TO CDK4/6 INHIBITOR PLUS LETROZOLE IN PARTICIPANTS OVER 18 YEARS OF AGE WITH HORMONE RECEPTOR (HR)-POSITIVE, HER2-NEGATIVE ADVANCED/METASTATIC BREAST CANCER WHO HAVE NOT RECEIVED ANY PRIOR SYSTEMIC ANTICANCER TREATMENT FOR ADVANCED/METASTATIC DISEASE (FOURLIGHT-3)}}

12. Clinical Framework (Breast Cancer Specific)

Disease Area/Name: {{Breast Cancer}} **Primary Condition:** {{HR-positive, HER2-negative Advanced/Metastatic Breast Cancer}} **Preferred Name:** {{Malignant neoplasm of breast}} **Preferred UMLS Name:** {{Breast Neoplasms}} **Semantic Type:** {{Neoplastic Process}} **MeSH Code:** {{D001943}} **SNOMED ID:** {{254837009}} **Condition Definition:** {{A primary or metastatic malignant neoplasm involving the breast. The vast majority of cases are carcinomas arising from the breast parenchyma or the nipple. Malignant breast neoplasms occur more frequently in females than in males.}} **Diagnosis Threshold:** {{Histological confirmation, ER and/or PR positive, HER2 negative}} **Medical Methodology:** {{Targeted therapy combined with Endocrine Therapy}} **Optimization Target:** {{Improvement in Progression Free Survival (PFS) and Overall Survival (OS)}}

13. Study Procedures & Methodology

Management Pathway: {{First-line treatment of Advanced/Metastatic Breast Cancer}} **Education Protocol (HCP):** {{Site Initiation Visits, protocol training, and AE management guidelines}} **Education Protocol (Patient):** {{Informed consent process, dosing diaries, and self-monitoring materials}} **Quality Audit Mechanism:** {{Blinded Independent Central Review (BICR) for tumor scans and continuous source data verification}}

14. Observation & Follow-Up Schedule

Total Duration: {{Up to approximately 13 years for survival follow-up}} **Onsite Visit Cadence:** {{Regular schedule aligned with 28-day treatment cycles}} **Remote Monitoring Frequency:** {{As specified in the clinical monitoring plan}} **Checklist Review Interval:** {{Every treatment cycle}}

Role	Name	Signature	Date		:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]					Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]					